A/PROF CHAMARA BASNAYAKE
MBBS, PhD, FRACP
Gastroenterologist & Endoscopist
PN 421926PK

FOCUS GASTROENTEROLOGY

16 April 2026

Dr Mark Benton
Guardian Medical

Dear Dr Benton,

Re: Mr Kerrod Bowman
DOB: 1991-04-19
Address: 3 Narraweena Court, BUNYIP, VIC, 3815

Summary

- **Significant improvement in bowel frequency attributed to pelvic floor physiotherapy.**
- **CT scan of the abdomen unremarkable for small bowel pathology.**
- **Serology negative for autoimmune markers including vasculitis and systemic lupus erythematosus.**
- **Ongoing management with pelvic floor physiotherapy and osmotic laxatives.**
- **No action required by you.**

I had the pleasure of conducting a telehealth review with Kerrod Bowman. You will be pleased to know he has reported a significant improvement in his bowel habit. He is now opening his bowels almost daily, which is a marked progress from our previous review in March where he was only achieving bowel movements three days per week.

Kerrod has commenced a programme with pelvic floor physiotherapy. He is currently focusing on initial techniques to facilitate pelvic floor relaxation and improve defaecatory mechanics, including posture and breathing techniques. I have encouraged him to continue with this, as patients with a hypertonic pelvic floor often require several months of therapy to achieve sustained results. He also continues to utilise osmotic laxatives, including Osmolax and Movicol, to assist with stool consistency.

Regarding his investigations, Kerrod's recent blood tests did not reveal any evidence of autoimmune illness; specifically, we have excluded vasculitis and systemic lupus erythematosus.

All appointments and enquires
Suite 201, 100 Victoria Parade
East Melbourne VIC 3002
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Email: office@focusgastro.com.au

Endoscopy locations
Epworth Freemasons Day Proc. Centre
320 Victoria Pde, East Melbourne, 3002
Hobson Healthcare Sydenham
566 Melton Hwy, Sydenham, 3037
St Vincent's Private Hospital
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During our consultation, we were able to review his recent CT scan performed at Imaging Associates. While the scan was performed without oral contrast—which can occasionally limit the detection of subtle small bowel narrowings or strictures related to Crohn's disease or NSAID use for his ankylosing spondylitis—the imaging was unremarkable for any small bowel pathology.

Impression and Plan The positive clinical response to pelvic floor therapy reinforces our hypothesis that his symptoms are primarily driven by pelvic floor dyssynergia rather than structural enteropathy. I have advised Kerrod to continue his sessions with Annabelle the physiotherapist and to maintain his current routine. We discussed his previous use of prucalopride; while he is currently using it intermittently with some success, I have noted that it is typically more effective when taken regularly, though I am comfortable with his current approach if it provides symptomatic relief.

I have warned Kerrod that his progress may not be strictly linear and that he may experience occasional setbacks in his bowel habit, particularly during periods of stress. We have agreed to a follow-up review in six months to ensure his symptoms continue on an upward trajectory.

Yours sincerely,

A/Prof Chamara Basnayake
Gastroenterologist

CC: Ms Annabelle Citroen

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